

PATSOGOLO CP FOUNDATION

Historical Growth Summary (2020–2026)

Care, inclusion and empowerment for children with CP, their families and communities

A summary of the organisation's evolution from a single hospital-based clinic to a multi-site foundation preparing for NGO registration.

Overview

Patsogolo CP Foundation traces its roots to a single physiotherapy-led clinic for children with cerebral palsy (CP) at Mangochi District Hospital. Over six years, it has grown into a four-to-five-site, multidisciplinary programme spanning clinical care, caregiver empowerment, health worker training, assistive device production, research, and national advocacy — and is now formalising as a registered NGO to sustain and scale this work.

Phase 1: Origins (2020–2022) — A Physiotherapy Clinic

- CP care in Mangochi began as a physiotherapy-only service, run out of Mangochi District Hospital and reaching two outreach sites.
- Cerebral palsy was recognised as the leading cause of childhood physical disability in the district, but care was fragmented: no integrated approach existed for the medical, nutritional, and developmental needs of these children.
- Community understanding of CP was — and remains, in parts — shaped by stigma and misconception (CP frequently attributed to curses or “something gone wrong,” per 2023 community interviews).

Phase 2: The Multidisciplinary Pilot (February 2023)

This is the pivotal moment in the organisation's history: the clinic was redesigned as a multidisciplinary outpatient service, combining physiotherapy, nutrition, medical care, and health education under one roof — modeled on the ICF (International Classification of Functioning, Disability and Health) framework.

Key baseline findings from the 2023 pilot (Reference 2022.0259)

- 42% of attending children had epilepsy; 40% had moderate-to-severe malnutrition (compared to 15–18% in the general child population of Mangochi); over 85% had swallowing difficulties.
- **Attendance increased 59%** following the shift to integrated care and community training.
- 14 health centres and 2 community hospitals were trained (1 Clinical Officer/Medical Assistant + 4 Health Surveillance Assistants each).
- Community health education sessions reached 73 stakeholders (chiefs, counsellors, religious leaders, teachers, traditional healers) across 5 villages, surfacing widespread stigma and the financial/isolation burden on caregivers — predominantly mothers and grandmothers.
- A validated Quality-of-Life questionnaire was administered to 62 patients, and 7 caregiver focus groups were held — early evidence-gathering that shaped later caregiver-support and dramaplay initiatives.
- Local device production began: 21 CP chairs, 21 standing frames, 19 rollators, 3 tricycles, 3 wheelchairs made with a local carpentry workshop.

Funding at this stage came through Cerebral Palsy Africa, with the project run under the KUHES Family Medicine Department and Mangochi District Hospital's Physiotherapy Department.

Phase 3: Scaling to Four Sites (2024)

The 2024 Wilde Ganzen proposal marked the shift from pilot to structured, multi-year programme, organised into 7 activity areas: health worker training/mentorship, continued integrated clinics, nutrition care improvement, home-based rehabilitation (via community rehabilitation volunteers), device production, caregiver support groups, and evaluation.

- **A third clinic site was added** at Malembo/Nankumba/Chilonga, bringing the total to the four-site model (Mangochi District Hospital, Malawi Children's Village, Njereza, Malembo) that has persisted since.
- **The EDACS (Eating and Drinking Ability Classification System) validation project began** — translating and validating an international feeding/swallowing assessment tool for the Malawian context (originally proposed to the Dutch Albert Schweitzer Foundation, co-funded with Wilde Ganzen; total budget €13,453).
- Nutrition interventions were formalised: Likuni Pala (local fortified porridge) for moderate malnutrition, RUTF referral pathways for severe cases.
- Home-based rehabilitation was piloted with 5 community rehabilitation volunteers, each supporting 4 families (20 families total).

Phase 4: Networks and Systems (2025)

2025 was a year of institution-building rather than just service expansion, under the Enablement Foundation/Wilde Ganzen partnership.

- **A CP/Neuro-Developmental Network meeting** was convened with ~50 participants from government, non-government, hospital, and community-based rehabilitation (CBR) organisations, covering early identification, clinical care, research, and advocacy.
- **Introduction of an electronic patient record system ("ReHapp")** to track follow-up, food supplement needs, and clinical data.
- The CP cookbook, in development since 2024, was finalised and printed.
- Early identification expanded into the maternity and nursery wards — reaching mothers of at-risk newborns directly at the point of birth (Mangochi Hospital sees ~1,200 deliveries/month).
- **Outreach began to Tiyende Pamodzi**, an existing rehabilitation-only provider, as a potential network/mentorship partner.
- Governance groundwork was laid for what would become Patsogolo CP Foundation, with a Board of Trustees and management team taking shape (Sandra Emons/Enablement as Chair; Alinafe Chisulanda as Executive Director; Alpheus Njewa and Bente van der Meijden in key roles).

Phase 5: Formalisation and Diversification (2026 to date)

2026 is the year the programme both formalised its identity as Patsogolo CP Foundation and diversified its clinical and research scope substantially.

Clinical expansion

- A 5th clinic site introduced at Monkey Bay Community Hospital (made possible by a newly placed rehabilitation technician there); Namwera considered as an alternative/additional site.

- **An Autism and ADHD clinic launched 28 April 2026**, following training of 9 health workers (physiotherapy, nursing, mental health, nutrition) by an occupational therapist — a new diagnostic category added to the CP programme's scope.
- Formal partnership established with MAP (Malawi Against Physical Disabilities) to train local artisans and improve locally made assistive devices.
- Modified feeding utensils distributed to 25+ children so far in 2026, shared with the CP clinic at Queen Elizabeth Central Hospital (QECH).

Research and publication milestones

- The CP cookbook was published in Field Exchange (issue 77, 2026): “Developing a practical cookbook for children with cerebral palsy in Malawi” — subsequently adopted into UNICEF's disability feeding resource bank, with interest from health workers in Tanzania and India.
- The validated Malawian Cerebral Palsy Quality-of-Life (CP QoL-Child) questionnaire was accepted for publication in the Malawi Medical Journal (July 2026) — the first tool of its kind validated for the country.
- A study on disability prevalence in malnutrition wards, and research into caregiver/health-worker experiences of accessing care, are ongoing.

Caregiver empowerment

- The caregiver-led dramaplay group grew to 12 members (including caregivers from Malembo), trained by Together Act Now, producing monthly physical and radio performances.
- Caregivers moved from passive participants to active co-creators of health education materials, tailored to their literacy levels and needs.

Networks and advocacy

- The Neuro-Developmental Disorders Network was formalised with KUHES Rehabilitation Department, QECH, Zomba Central Hospital, and Fount for Nations — positioning the group as a national resource/training centre for NDDs in Malawi.
- Monthly radio programming now reaches 8 districts, broadcast twice weekly and archived as a podcast on Spotify; a caregiver-developed radio drama was added in 2026.
- School inclusion work expanded: 2 private daycare centres and 1 government primary school assessed and given basic teacher training; an advisory role established at a newly opened special-needs daycare in Mangochi Boma.

Governance and sustainability

- Patsogolo CP Foundation formally requested a recommendation letter from the Ministry of Gender, Community Development and Social Welfare (July 2026) to support registration with the NGO Regulatory Authority.
- A detailed Year 1 Plan of Activities (August 2026–July 2027) was submitted alongside this request, setting targets across all seven programme areas.
- Funding constraints tied to the lack of a project bank account (a direct consequence of not yet being a registered NGO) forced cancellation of some 2026 activities — including wheelchair provision to 11 identified children and a planned MAP artisan training — underscoring the practical urgency of registration.
- Local fundraising has begun (e.g., a donation from Dossan Trust, sales of the cookbook and feeding devices) alongside Wilde Ganzen fundraising training for team members.

Growth at a Glance

Year	Milestone	Scale
2020–2022	Physiotherapy-only CP service	2 outreach sites
Feb 2023	Multidisciplinary clinic pilot launched	~200 children; 59% attendance increase
2023	Health worker & community training	14 health centres, 2 community hospitals, 73 community stakeholders
2024	3rd clinic site added (Malembo); EDACS project begins	4 clinic sites
2025	CP Network formed; electronic patient record introduced; cookbook printed	~300 children; ~50 network participants
2026	5th site (Monkey Bay); Autism/ADHD clinic; two journal publications; NGO registration submitted	300+ children; national/international research recognition

The Throughline

Each phase built directly on evidence gathered in the one before it: the 2023 baseline data on malnutrition and epilepsy justified the multidisciplinary model; caregiver focus groups from 2023–2024 shaped the dramaplay and co-creation approach used today; and the practical funding barriers experienced in 2025–2026 (no bank account, cancelled activities) are the direct rationale for formal NGO registration now underway. What started as a single physiotherapy clinic has become a foundation with a validated research base, national academic partnerships, and a seven-pillar programme model — while remaining rooted in the same core team (Bente van der Meijden, Alinafe Chisulanda, Alpheus Njewa) and the same district-level, community-embedded approach it began with.